Document Control

Title

Nerve Blocks For Pain Relief in Trauma
Standard Operating Procedure

Author's job title
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Directorate	Department	Team/Specialty
Clinical Support Services	Anaesthetics	Acute Pain

Version	Date Issued	Status	Comment / Changes / Approval
0.1	Aug 2015	Draft	Initial version for consultation
1.0	Sep 2015	Final	Approved by Anaesthetic Lead Clinician and Clinical Director Clinical Support Services
2.0	Jan 2018	Final	It's been reviewed and the only amendment is that the hyper-link to the Acute Pain Guidelines needs to be updated to match the current version

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Document Class	Target Audience
Standard Operating Procedure	Clinical Staff
Distribution List	**Distribution Method**
Senior Management	Trust's internal website

Superseded Documents: None

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January 2018	January 2021	Three years

Consulted with the following stakeholders:
- Anaesthetic Consultants
- Intensive Care Consultants
- Orthopaedic Surgeons
- Emergency Department Consultants
- Theatre and Recovery Staff
- Trauma Ward Managers

Contact responsible for implementation and monitoring compliance:
Acute Pain Lead

Education/ training will be provided by:
Anaesthetic Consultants
Emergency Department Consultants

Approval and Review Process
- Anaesthetic Lead Clinician

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Policy categories for Trust's internal website (Bob) Acute Pain	**Tags for Trust's internal website (Bob)** Acute Pain

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1. Background

All patients with painful fractures should be considered as to whether a nerve block is appropriate and feasible; this particularly applies to the elderly with fractured necks of femur. NICE guidelines on the management of fractured neck of femur advise consideration of a nerve block in addition to regular paracetamol and careful use of opioids if pain is difficult to control or opioid sparing is required.

Nationally accepted guidelines on standards of monitoring for anaesthesia, including nerve blocks, mandate that patients be monitored with a minimum of SpO2, NIBP and ECG during the procedure. It is also expected that full resuscitation facilities, trained assistance and IV access be available. In addition, steps need to be added to ensure that the block is performed at the correct anatomical site.

2. Purpose

2.1. The Standard Operating Procedure (SOP) has been written to:

- Ensure that standard analgesia is prescribed and administered in all patients with painful injuries and where appropriate, consideration is given to administering a nerve block to improve this.
- Ensure that all nerve blocks for pain relief in trauma are performed in an environment which allows the required standards of monitoring (SpO2, NIBP, and ECG), trained assistance and resuscitation equipment. Nerve blocks are to be performed by appropriately trained Medical personnel.
- Ensure that nerve blocks are always performed at the correct anatomical site, having been clearly identified by the team diagnosing the fracture and checked at the time of blockade. Formal consent is part of this process.

3. Scope

3.1. This Standard Operating Procedure (SOP) relates to the following staff groups who may be involved in the assessment and delivery of pain relief in trauma care in the Emergency Department (ED), on the wards and in Theatres / Recovery:

- Registered nurses
- Operating Department Practitioners
- Medical staff in Emergency Medicine, Orthopaedics and Anaesthetics

3.2. Registered nurses, medical staff and those trained in trauma-care by the Emergency Medicine and Orthopaedic Departments should assess the patient's need for nerve blockade as pain relief and make an informed decision as to whether a referral to the Anaesthetic Department is necessary.

4. Location

4.1. This Standard Operating Procedure regarding Nerve Blocks for Pain Relief in Trauma can only be implemented in clinical areas where competent members of staff are available to undertake this role and the required levels of patient monitoring and resuscitation facilities are available. In practice this is limited to the Emergency Department, Critical Care and the Theatre suites including Recovery.

4.2. Staff undertaking this procedure must be able to demonstrate continued competence and therefore, in practice, only appropriately trained medical staff from the Anaesthetic and Emergency Departments will perform the procedures.

5. Equipment

Monitoring: A minimum of blood pressure, SpO2 and ECG are required during the procedure and for 15 minutes after.

A trained assistant must be present.

Full resuscitation equipment must be available and intravenous access must be in situ. By performing the procedure in ED, ITU or Theatres / Recovery this will be covered. Intralipid 20% is kept in the theatres corridor on level 3 and on level 2 in day case recovery.

Ultrasound guidance is encouraged but not mandated if another, well accepted technique is used.

6. Procedure

Patient selection: Nerve blockade is an analgesic option ‘in addition to’ rather than ‘in place of’ standard analgesia. Patients must be prescribed routine analgesia which must not be withheld in anticipation of a nerve block. In the case of fractured hips, NICE guideline CG 124 is to be followed using regular paracetamol plus careful use of opioids. Nerve blocks are to be considered for opioid sparing and to improve quality of analgesia when opioids and paracetamol are not controlling the pain without excessive side effects.

Referral process: The decision to perform a nerve block in the Emergency Department is responsibility of the Emergency Department doctors who will either perform the block themselves or liaise with the on-call anaesthetic team to assist. Patients on the wards are best referred directly from the orthopaedic medical team but can be referred by nursing staff after documented consultation with the responsible orthopaedic team if they are unavailable to refer in person.

Consent: This is potentially a very beneficial but non-essential procedure with potential complications. Therefore documented consent is to be obtained and patients being taken to Theatres / Recovery will be expected to provide a consent form as part of their check list. Patients lacking capacity to give consent require ‘best interest’ decisions in line with the Mental Capacity Act and the appropriate documentation completed by the medical team in charge of the patient.

The affected limb is to be marked by the team diagnosing the fracture to ensure correct laterality. The affected side must also be clearly annotated in the notes. The mark should be an arrow in the manner used for surgical marking. In the case of multiple injuries where this may cause confusion it is permissible not to mark the patient. In which case, bedside handover must occur directly from the referring doctor to the doctor performing the block who will mark the patient with a large dot. The referring doctor should also make a specific entry in the notes to confirm. It is advised that the blocking doctor view imaging to ensure this correlates.

WHO check list use is encouraged in all areas and required in Theatres. During the confirmation of consent and laterality, the block site should be marked with a large dot if the arrow is not close to the injection point. The site is to be confirmed immediately before starting the block as part of the 'Stop Before You Block' routine.

Nerve blocks should be performed using an adequate level of asepsis – the site cleaned with chlorhexidine or similar solution and sterile gloves worn. Ultrasound probes must be covered, either with a full probe cover or a sterile transparent dressing such as the IV 3000. If a catheter is to be inserted then sterile technique is required.

Intravenous access and monitoring as previously stated must be maintained throughout the procedure and monitoring continue for at least 15 minutes after.

All routinely accepted techniques for performing nerve blocks (ultrasound guided or nerve stimulator for femoral, land marks for fascia iliaca) are acceptable. However ultrasound is always encouraged.

The check list attached is to be completed and kept in the notes for audit purposes.

7. References

NICE guideline 124, The Management of Hip Fracture in Adults:

http://www.nice.org.uk/guidance/cg124/chapter/1-recommendations

Association of Anaesthetist of Great Britain and Ireland (AAGBI) Recommendations For Standards of Monitoring During Anaesthesia and Recovery 4th Edition:

http://www.aagbi.org/sites/default/files/standardsofmonitoring07.pdf

Royal College of Anaesthetists 'Stop Before You Block' campaign:

www.rcoa.ac.uk/sites/default/files/CSQ-PS-sbyb-supporting.pdf

8. Associated Documentation

- Northern Devon Healthcare NHS Trust Policies for :
- Acute Pain Management Guidelines
- Check list for performing nerve blocks for pain relief in trauma (Page 7)

NERVE BLOCKS FOR PAIN RELIEF IN TRAUMA CHECK LIST

APPROPRIATE ENVIRONMENT

Theatre / Recovery / ITU / ED **Trained Assistant** ☐ **Monitoring** ☐

Patient Name ☐ **Patient ID No.** ☐ **Patient DoB** ☐

Injury marked by diagnostician ☐ **Site of block marked by blocking clinician** ☐

Imaging correlates with marking ☐

CONSENT

Failure ☐ **Infection** ☐ **Haematoma** ☐ **Nerve Injury** ☐ **IV toxicity** ☐

Mental Capacity Act Best Interest ☐

STOP BEFORE YOU BLOCK ☐

TECHNIQUE

Sterile Gloves ☐ **Chlorhexidine** ☐ **Probe Covered** ☐

Sterile Technique (if Catheter) ☐ **Ultrasound** ☐ **Nerve Stimulator** ☐

Local Anaesthetic: Drug **Strength** **%** **Volume** **mls**

COMPLICATIONS

Yes ☐ No ☐

If 'Yes' have they been documented in patient's notes? Yes ☐ No ☐

Name: Signature: Date: